



## **WINDCALM MIXTURE 40mg/ml**

*Description: White coloured and anise flavoured suspension*

**Content:** Light magnesium carbonate..... 200mg / 5ml

### **Indications:**

For constipation, indigestion and hyperacidity.

### **Pharmacology:**

Magnesium carbonate reacts chemically to neutralize or buffer existing quantities of stomach acid but has no direct effect on its output. This action results in increased pH value of stomach contents, thus providing relief of hyperacidity symptoms. It also has laxative actions.

About one-third of magnesium is absorbed from the small intestine after oral doses and even soluble magnesium salts are generally very slowly absorbed. The fraction of magnesium absorbed increases if magnesium intake decreases. In plasma, about 25 to 30 % of magnesium is protein bound. Oral doses are eliminated in the urine (absorbed fraction) and the feces (unabsorbed fraction). Small amounts are distributed into breast milk. Magnesium crosses the placenta.

### **Adverse reactions:**

Chalky taste, stomach cramps, flatulence, diarrhoea, nausea and vomiting may occur occasionally.

### **Contraindications:**

It should be contraindicated in patients with severe renal function impairment, undiagnosed gastrointestinal or rectal bleeding and intestinal obstruction.

### **Precautions:**

Caution in patients with renal impairment, appendicitis, gastrointestinal or rectal bleeding, chronic diarrhea and intestinal obstruction. Risk of hypermagnesaemia in very young children, especially in dehydrated children or in those with renal failure. Therefore, use in children below 6 years is as directed by physician. The maximum dosage recommended should not be taken for more than 2 weeks, unless otherwise directed by physician.

### **Use In Pregnancy And Lactation:**

Animal studies are insufficient with respect to effects on pregnancy, embryonal / foetal development, parturition and postnatal development. The potential risk for humans is unknown. As there is no specific data for this product, it is advised that all pregnant and breast feeding women should consult their physician before using this product.

### **Drug Interactions:**

Antacids may interact with numerous other drugs, affecting the rate and extent of their absorption and in some cases their renal elimination. Changes in gastric pH affect the dissolution of the other drugs, and together with altered gastric emptying can markedly influence absorption. Several mechanisms may play a part in any particular interaction. In order to minimize the risk of interactions, this product should not be taken within two to four hours of other oral medications (allow at least 4 hours before or 2 hours after erlotinib). Examples of medications which may be affected include, but are not limited to ACE inhibitors, salicylates e.g. aspirin, atazanavir, azithromycin, barbiturates, bile acids, bisphosphonates, cephalosporin antibiotics, fluoroquinolone antibiotics, chloroquine and hydroxychloroquine, deflazacort, digoxin, dipyridamole, eltrombopag, erlotinib, fexofenadine, gabapentin, iron preparations, isoniazid, itraconazole, ketoconazole, lansoprazole, levothyroxine, lithium, methenamine, mycophenolate, nitrofurantoin, penicillamine, phenothiazines, phenytoin, proguanil, ritampicin, rosuvastatin, sulpiride, tetracyclines, tipranavir, ulipristal (avoid use with antacids). Concurrent and prolonged use with milk or milk products or calcium containing preparations may result in the milk alkali syndrome. There is a risk of metabolic alkalosis when oral magnesium salts are given with polystyrene sulphonate resins.

### **Dosage And Administration:**

#### As an antacid

Adults: 1-2 teaspoons three or four times a day, after meals.

6 years and over: ½ -1 teaspoon three or four times a day, after meals.

Below 6 years: As directed by physician.

Shake bottle well before use.

#### As a laxative:

Adults: 1-2 tablespoons three or four times a day, after meals.

6 years and over: ½ -1 tablespoon three or four times a day, after meals.

### **Symptoms and Treatment of over dosage:**

Overuse of antacids may inhibit digestive enzyme activity of pepsin, which is ineffective when the stomach contents reach a pH of 4.0 and above. Overdose, excessive or prolonged intake of magnesium containing antacids may give rise to hypermagnesaemia, especially in patients with renal insufficiency. Symptoms of hypermagnesium include nausea, vomiting, flushing of the skin, thirst, drowsiness, hypotension, confusion, muscle weakness, CNS and respiratory depression, hyporeflexia, peripheral vasodilatation, bradycardia, cardiac arrhythmias, coma and cardiac arrest.

Treatment of mild hypermagnesium is usually limited to restricting magnesium intake. In severe hypermagnesaemia, ventilatory and circulatory support may be required. Treatment should consist of the intravenous administration of calcium gluconate injection 10% at a dose of 10-20ml, to counteract respiratory depression or heart block. If renal function is normal, adequate fluids should be given to assist magnesium removal from the body. Haemodialysis may be necessary in patients with renal impairment or for whom other methods prove ineffective. Symptomatic and supportive measures should be provided as well.

**Storage Conditions :** Store in a cool, dry place below 30° C. Keep out of reach of children. Protect from light.

**Presentation :** 250ml/bottle

**Shelf life :** 3 years

**Manufactured & Distributed by :** **PRIME PHARMACEUTICAL SDN. BHD.**

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